جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سحر ابراهيم الذكير محمد
المحافظة : الجيزه **الحي / المدينة :** ابوالنمرس **العنوان :** منيل شيحه
رقم البطاقة : 28208251500342 **رقم التليفون :** 01002623269 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 25-08-1982 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني وزولاديكس [14165] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سحر ابراهيم الذكير محمد
National ID: 28208251500342

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Hormonal therapy including goserelin (Zoladex).

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front
[[DOCTYPE:ID_FRONT]]

MDT form

A-Patient ID:
1- Name : [redacted] Sex: Female
3- Medical number: 83263181
Old Med : 625-2025
2-ID number: 28208251500342
Patient Age : 43 Y

Family history
Phone: 01022936369
BMI
REASON OF MDT
marital status
NRS
GROUP
Date of MDT 2025-05-26
Co morbidity
Menopausal status
Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :
ct cap 3/2025: few enhancing rt HFL
triphasic 4/2025:stationary course of HFL
stationary L1,L2,L5 mixed lytic & sclerotic
no pulmonary nodules

BX	ER	PR	Her 2 Neu	SISH	Ki67
	8/8	8/8	1		15

T	N	M

Sonomammography

PATIENT HISTORY
ct cap 3/2025: few enhancing rt HFL
triphasic 4/2025:stationary course of HFL
stationary L1,L2,L5 mixed lytic & sclerotic
no pulmonary nodules

Bone Scan 6/2025: Free

MDT OUTCOME & PLAN

for zoladex+ femara +zometa 3/months

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: سحر ابراهيم الزكير	
MR. : 4725	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 43-year-old individual with de novo metastatic breast cancer diagnosed in March 2025, with hepatic metastases. A bone scan performed in June 2025 demonstrated disease involvement. Pathology of the primary invasive ductal carcinoma (IDC) was grade II, estrogen receptor (ER) score 8/8, progesterone receptor (PR) score 8/8, HER2 + 1, and Ki-67 15 %. The patient was started on anastrozole (FEMARA) and leuprolide acetate (ZOLADEX) in July 2025. A CT of the chest, abdomen, and pelvis on 24 November 2025 showed regression of the left breast mass. A thyroid nodule was identified and recommended for ultrasound correlation. The left hepatic segment VI lesion measured 9 mm on imaging. Laboratory workup was ordered, and the patient received FEMARA, calcium supplementation (1 × 1 × 90), and ZOLADEX 10.8 mg on 27 April 2026.

The patient continued on FEMARA with supportive care and underwent bilateral thyroid ultrasound, which demonstrated bilateral thyroid nodules: TIRADS 3 in the left lobe and TIRADS 2 in the right lobe. The findings were noted for supportive management and potential surgical consultation.

Supportive care with FEMARA was continued for the next two months, with the same bilateral thyroid nodule findings (TIRADS 3 left, TIRADS 2 right) pending surgical evaluation.

The treatment regimen was augmented with zoledronic acid (Zometa) and continued leuprolide acetate (ZOLADEX) for two months; laboratory results were pending.

The patient remained on FEMARA with supportive care for three months, with the next leuprolide acetate (ZOLADEX) dose scheduled for 23 July 2026. The next clinic visit was planned for 3 September 2026.

A CT chest, abdomen, and pelvis performed on 22 June 2026 showed left breast density measuring approximately 10 × 8 mm. Axillary lymph nodes retained preserved hilum, measuring about 19 × 11 mm. Breast and axillary ultrasound were ordered for further assessment. The patient received ZOLADEX 10.8 mg, continued FEMARA, calcium supplementation, bone-protective care, and was prescribed panadol for three days starting 11 August 2026. The current plan includes ongoing FEMARA, ZOLADEX, bone care, and follow-up imaging as indicated.

تحريرا في ,

التاريخ :	18-09-2026	**الوقت :**	

الطبيب المعالج : مايكل جمال فوزى

يعتمد ,

Patient Name	: الاستاذه / روديا فؤاد عبدالسميع سيد احمد	Request No.	: 13005394
Patient No.	: 29207130103742	Request Date	: 04/07/2022
Path No.	: 1917/22	Verification Date	: 26/07/2022
Sex	: Female	Financial Category	: برنامج نادي السلام - بطاقة الدولة
Age	: 29 Y	Department	: Out Patient
Refereing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:
Right breast invasive carcinoma and left breast suspicious microcalcification.

Nature of the specimen: Left wired wide local excision, Right retroareloar of NSM and right nipple sparing mastectomy specimen.

Gross Examination: Three specimens were received:
1- Left wired wide local excision measured 6x5 cm with skin covering measured 2.5cm. Cut section showed no gross tumours.
margins were negative.
2- Right retroareloar of NSM: measured 3x2cm.
Cut section showed no gross tumours. No malignancy.
3- Right nipple sparing mastectomy specimen measured 20x12x4cm with no skin covering.
Axilla measured 8x3x1cm.
- Tumor extent
  - o Number: Cut section revealed ill defined grayish white ruberry tumour bed site.
  - o Size: measured 3x3x3cm.
- Distance from margins: away 0.1cm from deep margin (inked).
- Lymph nodes number and size: Axillary dissection revealed 20 nodes up to 2x1cm.
- Frozen section: No malignancy and negative margins.

Microscopic examination of right breast (Right retroareloar of NSM and Right nipple sparing mastectomy specimen):
- Histolcgical type: Complete pathological response in previously diagnosed carcinoma.
- Angioinvasion: No.
- Ductal carcinoma in situ: No.
- Lymph node involvement: Free (0/20).
- Resection Margins: Free.

Verified by :
DR.Menna Nabil

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الاستاذه / روديا فؤاد عبدالسميع سيد احمد	Request No.	: 137894
Patient No.	: 29207130103742	Request Date	: 21/11/2021
Path No.	: SR-654/21	Verification Date	: 27/11/2021
Sex	: Female	Financial Category	: اورام دار السلام - ثقة الدولة
Age	: 29 Y	Department	: Out Patient
Referreing Doc :			

Pathology Report

Clinical Data:
Right breast canc

Nature of the specimen:
Slide revision

Gross Examination:
One paraffin block labeled as ((8946) and Three slides were received.

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): II
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: N/A
- Resection Margins: N/A
- Histologic response to therapy: N/A
- Additional pathologic findings: N/A

Diagnosis:
Right breast cancer; Slide revision:
- Invasive duct carcinoma, nuclear grade II
- Biomarkers:
o ER: Intensity 70% nuclear reaction (Allred score 7/8)
o PR: Intensity 70% nuclear reaction (Allred score 7/8)
o Her2: Positive for complete intense membranous stainig in >10% of tumor cells (score 3+)

Prof.Dr. Gamal Abdel Atty
Dr. Walaa Ghonaim

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سحر ابراهيم الزكير	Request No.	: 13004911
Patient No	: 28208251500342	Request Date	: 18/01/2025
Lab No.	: 221/25	Verification Date	: 06/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 18/01/2025.

Gross Examination:
Three grayish white tissue cores up to 1.5 cm, totally submitted.
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
Histological type: Invasive duct carcinoma.
Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
Ductal carcinoma in situ: Positive.
Perineural invasion: Not detected.
Angioinvasion: Not detected.
Microcalcifications: Not detected.
IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
Her2: Weak incomplete membrane reaction in <10% of tumor cells (score 1+).
Ki67: 15% nuclear reaction of tumor cells.

Diagnosis:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with DCIS.

ER: Positive (score 8/8).
PR: Positive (score 8/8).

Verified by :
Dr.Toka Medhat

Printing Date : 12-2-2025 11:30:03 Page 1 of 2 Printing By : .marwa mohamed youss

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sahar Ibrahim Alzkir Mohamed	Date : 20/04/2025
Exam : TRIPHASIC ABDOMEN &PELVIS	Age : 42 Year(s)
Medical ID: 625/2025	Pat. ID: 98379

Post-Contrast multi-slice CT scan of the chest, abdomen and pelvis with the liuver imaged in triphasic pattern:

- **Reported case of left breast cancer compared to previous study at 13-3-2025**

I Chest:

- **Progressive course** regarding size of previously noted Left breast lower outer domain multiple irregular mass lesions the largest is measuring curreritly 3x3 cm.
- **Still noted** rather unchanged Prominent subcentimeteric left supraclavicular lymph node measuring 8 mm.
- No sizable suspicious pulmonary nodules, masses, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.
- **Stationary** Left thyroid hypodense nodule for ultrasound correlation.

II Abdomen & pelvis:

- Average sized liver showing **stationary course** regarding size of preivously noted few right hepatic lobe enhacning focal lesions the largest is at segment VI, it still measures 9 mm shows arterio-portal enhancement with washout in delayed phase .
- Patent homogeneous portal vein as well as its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Stationary Heterogenous osseous texture evident by by few lytic and sclerotic lesions more evident at L1 and L2 lumbar vertebrae.

Opinion:

Reported case of left breast cancer revealed:

- **Stationary course** regarding size and number of Few enhancing right hepatic lobe focal lesions, Likely metastatic
- **Stationary L1, L2 and L5 vertebral mixed lytic and sclerotic lesions for bone scan assessment**
- **No sizable pulmonary metastatic deposits.**

Dr. Ahmed Elgazar
Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Patient's Name:	**Sahar Ibrahim Al-Dhikir**
Date:	**2026-08-27**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

EVCo

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: acase of pathologically breast cancer post neo adjuvant chemotherapy follow up study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.
- Few supra-clavicular lymph nodes are noted .

left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- regressive coarse of the previously noted spiculated masses showing UOQ area of architextural distorion .
- Regressive course regarding the previously reported malignant mass lesions seen at 2o'clock measuring 1.2x6mm (the largest) and 6mm the smaller one
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.
- Few supra-clavicular lymph nodes are noted .

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 6): Regressive course regarding the previously detailed malignant process as described

Best Regards,

DR.Dina sayed , M.Sc.CUDBIT

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Patient's Name:	**Sahar It kir**
Date:	**2026-08-27**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

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DR.Dina sayed , M.Sc.CUDBIT

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Mammogram

Patient's Name:	**Sahar It kir**
Date:	**2026-08-27**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

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